

NIC CPD Micro-Credential: Mental Health, Emotional Wellbeing & Psychosocial Support for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 5–6 CPD learning hours

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Author: Manus AI

Scope and safety statement: This is professional development for caregivers. It is not a psychology degree, psychotherapy, counselling, psychiatric nursing, clinical mental-health assessment or crisis-management qualification. It does not authorise caregivers to diagnose mental-health conditions, provide therapy, prescribe medication or manage psychiatric emergencies independently. Serious or immediate safety concerns require urgent professional or emergency support according to the applicable local procedure.

1. Course title

NIC CPD Micro-Credential: Mental Health, Emotional Wellbeing & Psychosocial Support for Professional Caregivers

2. Course overview

Professional caregiving includes more than physical tasks. People receiving care also have emotional wellbeing, relationships, identity, dignity, autonomy, belonging, values and cultural identity. This 5–6 hour CPD develops the caregiver's ability to provide respectful, person-centred psychosocial support within role.

The course covers psychological safety, active listening, supportive communication, loneliness, social connection, meaningful activity, grief, adjustment, anxiety awareness, low-mood awareness, trauma-informed principles, boundaries, confidentiality, safeguarding, crisis recognition, documentation and escalation. It uses person-first language and treats behaviour as information rather than a character judgement.

Behaviour is information, not a character judgement.

The course complements NIC’ s full caregiver programmes and Dementia Care CPD. It does not become a second dementia course or a substitute for clinical mental-health care.

Course at a glance

Element	Specification
Audience	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, supervisors, community-care workers and direct-care personnel
Duration	5–6 CPD learning hours
Structure	6 modules, 18 lessons, module assessments and a 35-question final assessment
Central skill	Respectful psychosocial support within caregiver scope
Final pass mark	80% (28/35)
Scenario emphasis	28 of 35 final questions are scenario/application based
Safety limitation	Online completion does not prove counselling, psychotherapy, psychiatric crisis-management or emergency-response competence

3. Strategic purpose of the CPD

Caregivers often notice changes in mood, participation, communication, sleep, appetite, relationships and behaviour before a formal review takes place. They may also be the person who listens when a client feels lonely, anxious, homesick, embarrassed, bereaved or uncertain about a new care arrangement. A professional response can create trust and connection; a dismissive, intrusive or over-involved response can increase distress and risk.

This CPD develops practical judgement. It teaches caregivers to connect, listen, observe, support, promote appropriate connection, maintain boundaries, document and escalate when necessary. It does not teach diagnosis. A caregiver may notice persistent withdrawal and report it; the caregiver should not conclude that the client “has depression.”

The course is relevant to Nigerian homes, home-care agencies, residential facilities, hospitals and community settings. It recognises multigenerational families, diaspora

relationships, religious communities, stigma, language diversity, financial stress, urban isolation, rural connection, disability, ageing and workforce pressures without stereotyping. Local safeguarding, emergency and referral contacts must be inserted before publication.

4. Target audience

The target audience is professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and other direct-care personnel who provide practical and emotional support to people receiving care.

The credential is not intended to qualify psychologists, psychiatrists, counsellors, therapists, psychiatric nurses, mental-health clinicians or crisis specialists. Those professionals require separate education, registration and supervision.

5. Recommended CPD hours

NIC should award **5–6 CPD learning hours**, based on lesson interaction, scenario work, module assessments, final assessment and review. A suggested allocation is 3.5–4 hours for lessons, 60–75 minutes for assessments and 45–60 minutes for final assessment and reflection.

6. Prerequisites

Learners should already understand basic caregiving, dignity, respectful communication, observation, professional boundaries, confidentiality, safeguarding, documentation and reporting/escalation. NIC may accept evidence of foundational caregiver training, the Elderly Care & Gerontology programme or current direct-care experience.

7. Learning outcomes

By completion, the learner should be able to:

1. Explain emotional wellbeing and psychosocial support in person-centred caregiving.
2. Create psychological safety through respectful, predictable and non-judgemental conduct.
3. Use active listening, empathy, reflection, clarification, silence and summarising without providing counselling.
4. Respond appropriately to sadness, fear, loneliness, frustration, anger, grief, uncertainty and loss of independence.
5. Recognise possible loneliness, isolation, anxiety, low mood and meaningful changes from baseline without diagnosing.

6. Promote safe social connection and meaningful activity without becoming a client's substitute family or primary friend.
7. Support grief and adjustment while respecting individual, cultural and faith practices.
8. Maintain professional boundaries, confidentiality and appropriate digital conduct.
9. Recognise safeguarding, self-harm, suicide and crisis concerns and escalate urgently when necessary.
10. Document objective observations and communicate concerns through the correct pathway.

8. Competency framework

Domain	Competent caregiver behaviour	Evidence
Emotional wellbeing	Recognises that care includes identity, belonging, autonomy and emotional needs	Module 1
Psychological safety	Uses predictable, respectful and non-humiliating conduct	Module 1
Listening and communication	Listens, acknowledges emotion and avoids premature advice	Module 2
Connection	Identifies safe, meaningful social and community opportunities	Module 3
Meaningful activity	Matches person, interest, ability, choice and safe participation	Module 3
Grief and adjustment	Provides presence, patience and appropriate escalation	Module 4
Mental-health awareness	Notices patterns and reports without diagnosing	Module 4
Boundaries and confidentiality	Maintains role clarity, privacy and digital professionalism	Module 5
Safeguarding and crisis	Protects safety, does not promise secrecy and escalates	Module 6
Documentation	Records facts, changes, responses, notifications and	Module 6

9. Psychosocial support response framework

The NIC CONNECT framework

C — Connect respectfully. Approach calmly, use the person's preferred name and create a sense of presence.

O — Open the conversation. Use a simple open question or observation: “You seem quieter today. How are you feeling?”

N — Notice and listen. Attend to words, emotion, behaviour, routine and changes from baseline. Allow silence.

N — Name and acknowledge emotion. Use validation without agreeing with unsafe beliefs or claiming clinical expertise.

E — Enable practical support. Offer choices, predictable routines, comfort, activity or appropriate connection within the care plan.

C — Connect onward. Support contact with authorised family, community, faith or professional resources where appropriate.

T — Track boundaries and facts. Maintain role clarity, confidentiality and objective documentation.

Urgent override: If there is immediate danger, suspected abuse, serious self-harm or suicide concern, threat toward others, severe confusion or inability to maintain safety, **PROTECT SAFETY → SEEK URGENT HELP → DOCUMENT → HAND OVER.** Do not manage serious risk alone.

10. Complete module structure

Module	Title	Lessons	Assessment
1	Emotional Wellbeing and Psychological Safety	1. Emotional Wellbeing in Care; 2. Psychological Safety; 3. Person-First, Culturally Respectful Care	6 questions
2	Listening and Supportive	4. Active Listening; 5. Supportive Responses;	6 questions

	Communication	6. Distress, Anger and Uncertainty	
3	Loneliness, Connection and Meaningful Activity	7. Loneliness and Social Isolation; 8. Meaningful Activity; 9. Relationships and Independence	6 questions
4	Grief, Adjustment and Mental-Health Awareness	10. Loss, Grief and Bereavement; 11. Adjustment to Care; 12. Anxiety, Low Mood and Change	7 questions
5	Boundaries, Confidentiality and Trauma-Informed Care	13. Professional Boundaries; 14. Confidentiality and Digital Conduct; 15. Trauma-Informed, Stigma-Aware Practice	7 questions
6	Safeguarding, Crisis Recognition and Escalation	16. Safeguarding and Serious Concern; 17. Self-Harm, Suicide and Crisis Awareness; 18. Documentation, Handover and Integrated Judgement	7 questions

11. Detailed lesson outline

Lesson	Primary purpose
1	Understand emotional and psychosocial wellbeing in caregiving
2	Build psychological safety through reliable conduct
3	Use person-first and culturally respectful language
4	Practise active listening without becoming a therapist
5	Respond supportively to common emotions

6	Manage distress, anger and uncertainty safely
7	Recognise loneliness and isolation
8	Plan meaningful activity around the person
9	Promote connection and independence without over-involvement
10	Support grief and bereavement
11	Support adjustment to care and change
12	Recognise anxiety, low mood and significant change without diagnosis
13	Maintain boundaries and recognise over-involvement
14	Protect confidentiality and use digital communication safely
15	Apply trauma-informed, stigma-aware practice
16	Recognise safeguarding and serious emotional concerns
17	Respond to self-harm, suicide and crisis signals
18	Document, hand over and apply integrated judgement

12. Full lesson content

Module 1 — Emotional Wellbeing and Psychological Safety

Lesson 1. Emotional Wellbeing in Care

Why this matters in professional caregiving

A person may receive excellent physical care and still feel lonely, frightened, powerless or unseen. Emotional wellbeing includes feeling respected, connected, heard, safe and able to make choices. Psychosocial support means practical, relational and social support that helps a person cope and participate; it is not psychotherapy.

Learning objectives

The learner should be able to describe emotional wellbeing; identify psychosocial needs; use person-first language; and recognise that behaviour can communicate a need.

Core instructional content

Emotional wellbeing is not constant happiness. People may feel sadness, anger, fear, grief, frustration or uncertainty and still receive respectful care. The caregiver's responsibility is not to remove every difficult emotion but to respond safely, respectfully and within role.

Identity matters. A person may be a parent, worker, farmer, professional, neighbour, faith member, artist or community leader as well as someone receiving care. Use the person's preferred name and avoid reducing them to a diagnosis, age, disability or care task.

Behaviour is information, not a character judgement. Withdrawal may signal fatigue, grief, pain, fear, depression, hearing difficulty, cultural discomfort or a wish for privacy. Irritability may signal distress. The caregiver observes and reports rather than labelling someone "difficult," "dramatic," "crazy" or "attention seeking."

Applying This in Practice

Ask what matters to the person: who they value, what routines provide comfort, which activities have meaning and what makes them feel safe. Nigerian family and faith relationships may be important, but never assume the person wants all relatives involved.

Case scenario

A client has stopped joining conversations and is labelled "uncooperative" by another worker.

Best response: Observe the change, speak privately and respectfully, consider possible contributors and document/report if persistent. Do not adopt the label.

Common mistakes / professional risks

Dismissing emotion, using diagnosis as identity, speaking about clients as if they are absent, assuming withdrawal is bad character and confusing support with treatment.

Key practice points

1. Emotional wellbeing is part of professional care.
2. Use person-first language.
3. Behaviour may communicate a need.
4. Observe patterns; do not diagnose.

5. Protect dignity and choice.

Lesson assessment

1. **MCQ:** Psychosocial support is best described as: A) psychotherapy; B) practical, relational and social support within role; C) diagnosis; D) medication management. **Answer: B. Objective: 1.**
2. **Scenario:** A client withdraws from conversation. **Answer:** Explore respectfully, observe and report meaningful change. **Objective: 5.**
3. **True/False:** “Difficult” is an objective description. **Answer: False. Objective: 3.**
4. **What should not happen?** Assume withdrawal is laziness. **Answer:** Consider multiple possible contributors. **Objective: 5.**
5. **Application:** Name three aspects of wellbeing. **Answer:** Belonging, identity, dignity, autonomy, relationships or meaning. **Objective: 1.**
6. **Scope:** Can a caregiver diagnose depression? **Answer: No. Objective: 5.**

Lesson 2. Psychological Safety

Why this matters in professional caregiving

Psychological safety grows when people know what to expect, are treated without humiliation and can express preferences or concerns without retaliation. Unpredictable, threatening, dismissive or intrusive caregiver behaviour can increase distress.

Learning objectives

The learner should be able to create predictability, trust, privacy and non-judgement; use consent and choice; and recognise conduct that increases distress.

Core instructional content

Psychological safety is supported by calm introductions, reliable routines, clear explanations, respectful touch, privacy, consent and honest boundaries. Explain what you are doing before doing it. Offer reasonable choices. If a plan must change, explain why and provide as much predictability as possible.

Avoid threats, humiliation, unnecessary confrontation, sarcasm, public correction and promises you cannot keep. Appropriate reassurance is realistic: “I will stay with you while I contact the supervisor,” rather than “Nothing bad will happen.”

A caregiver can be warm without becoming a personal friend. Reliability matters: arrive as agreed, follow through, acknowledge mistakes and communicate delays. If the person is

distressed, reduce stimulation and use a calm voice.

Applying This in Practice

In home care, explain who is entering the home and what will happen. In residential care, preserve familiar routines and privacy. In hospitals, introduce yourself before touching personal belongings or moving equipment.

Case scenario

A caregiver jokes publicly about a client's incontinence to make others laugh.

Best response: Stop the humiliating conduct, protect privacy, report the professional concern through the appropriate route and document if required.

Common mistakes / professional risks

Threatening compliance, public embarrassment, forced familiarity, ignoring consent, changing routines without explanation and promising secrecy.

Key practice points

1. Predictability supports safety.
2. Explain and seek consent where appropriate.
3. Use realistic reassurance.
4. Never humiliate or threaten.
5. Reliability is part of care.

Lesson assessment

1. **MCQ:** Psychological safety is strengthened by: A) threats; B) predictability, privacy and non-judgement; C) public correction; D) secrecy. **Answer: B. Objective: 2.**
2. **Scenario:** A client is distressed by an unexpected worker. **Answer:** Introduce yourself, explain, offer choice and allow time. **Objective: 2.**
3. **True/False:** Humour at a client's expense can build trust. **Answer: False. Objective: 2.**
4. **What should not happen?** Promise "I will never tell anyone." **Answer:** Serious safety information may require escalation. **Objective: 9.**
5. **Application:** Name three predictable behaviours. **Answer:** Explain actions, arrive reliably, respect privacy, communicate changes. **Objective: 2.**
6. **Scenario:** A plan changes unexpectedly. **Answer:** Explain, offer choices and maintain calm. **Objective: 2 and 4.**

Lesson 3. Person-First, Culturally Respectful Care

Why this matters in professional caregiving

Language shapes expectations. Stigma and stereotypes can discourage disclosure, reduce access to help and damage relationships. Culture, faith, family and community may be sources of strength, but they are not identical for every person.

Learning objectives

The learner should be able to use respectful language; identify cultural and communication preferences; avoid stigma; and include family or faith connection only with appropriate consent and authorisation.

Core instructional content

Use “a person living with...” or the person’s preferred language rather than labels. Avoid “crazy,” “unstable,” “manipulative,” “attention seeker,” “dramatic” and similar terms. Describe observable behaviour: “The client spoke less than usual and declined the activity.”

Ask about language, preferred form of address, faith, cultural practice, family roles, privacy and communication needs. Do not assume that a religious explanation, family decision or cultural practice is either helpful or harmful; ask the person and follow safeguarding and consent requirements.

Stigma may appear as gossip, exclusion, ridicule or the belief that emotional distress is weakness. Respond with respect and encourage appropriate professional support. Do not debate beliefs aggressively or promise that prayer, willpower or positive thinking will cure a mental-health problem.

Applying This in Practice

In Nigeria’s language-diverse settings, use an interpreter or trusted communication support where authorised. Do not use children as interpreters for sensitive information when a safer option exists.

Case scenario

A relative says the client is “just seeking attention” and asks staff to ignore the client’s sadness.

Best response: Continue respectful observation, listen to the client, document meaningful change and report concerns through the care pathway.

Common mistakes / professional risks

Stereotyping, gossip, dismissing faith or culture, using untrained interpreters, labelling behaviour and assuming family wishes replace client voice.

Key practice points

1. Language can protect or harm dignity.
2. Ask rather than assume.
3. Describe behaviour, not character.
4. Respect culture without abandoning safety.
5. Escalate concerning change.

Lesson assessment

1. **MCQ:** Which is objective? A) “Client is manipulative” ; B) “Client declined two activities and spoke less” ; C) “Client is crazy” ; D) “Client is dramatic.” **Answer: B. Objective: 3 and 10.**
2. **Scenario:** Family wants to exclude the client from decisions. **Answer:** Respect autonomy and follow consent/care policy. **Objective: 3.**
3. **True/False:** Every person in one faith community wants the same support. **Answer: False. Objective: 3.**
4. **What should not happen?** Promise that prayer alone will cure distress. **Answer:** Offer respect and appropriate referral, not unsupported treatment claims. **Objective: 5.**
5. **Application:** Name three cultural questions. **Answer:** Preferred language, faith practice, family involvement, privacy, routines or forms of address. **Objective: 3.**
6. **Scenario:** A client cannot communicate easily in the worker’s language. **Answer:** Arrange authorised communication support. **Objective: 3.**

Module 2 — Listening and Supportive Communication

Lesson 4. Active Listening

Why this matters in professional caregiving

Listening helps the person feel heard and gives the caregiver information about needs and risk. It is not counselling. The caregiver’s job is to be attentive, acknowledge emotion, clarify, provide practical support and escalate when needed.

Learning objectives

The learner should be able to use attentive presence, open questions, reflection, clarification, silence, summarising and acknowledgment.

Core instructional content

Active listening includes facing the person appropriately, reducing distractions, allowing pauses, asking open questions and checking understanding. Reflection may be simple:

“You are worried that the move means losing your independence.” Clarification prevents assumptions: “When you say you feel unsafe, what are you concerned about?”

Summarising confirms what was heard.

Avoid premature advice, false reassurance, changing the topic, arguing, interrupting or making the conversation about your own experience. “Don’ t worry” may sound dismissive. A more supportive response is: “That sounds like it has been weighing on you. Would you like to tell me more?”

If the content suggests abuse, self-harm, suicide, serious deterioration or danger, listening must lead to safeguarding and escalation. Do not promise secrecy.

Applying This in Practice

Use short conversations appropriate to the setting. If the person is tired or distressed, offer a pause and return at an agreed time. Record relevant facts, not every personal detail.

Case scenario

A client says, “I feel invisible here.” The caregiver wants to say, “At least you have a bed.”

Best response: Acknowledge the emotion and invite more: “That sounds lonely. What would help you feel more seen?”

Common mistakes / professional risks

Minimising, over-questioning, giving personal stories, offering therapy, promising secrecy and failing to act on a safety disclosure.

Key practice points

1. Presence communicates respect.
2. Allow silence.
3. Reflect emotion without diagnosing.
4. Clarify safety concerns.
5. Listen, then support or escalate.

Lesson assessment

1. **MCQ:** The best response to “I feel invisible” is: A) “Don’ t worry” ; B) “That sounds lonely. Tell me more” ; C) “Everyone feels that way” ; D) change topic. **Answer: B. Objective: 3–4.**
2. **Scenario:** The client pauses and cries. **Answer:** Allow silence, remain present and acknowledge emotion. **Objective: 3.**
3. **True/False:** Active listening requires the caregiver to provide counselling. **Answer: False. Objective: 3.**
4. **What should not happen?** Interrupt with your own story. **Answer:** Keep the focus on the client. **Objective: 3.**
5. **Application:** Name four active-listening skills. **Answer:** Open questions, reflection, clarification, silence, summarising, acknowledgment. **Objective: 3.**
6. **Scenario:** The client says they are unsafe. **Answer:** Clarify enough to protect safety and escalate. **Objective: 9.**

Lesson 5. Supportive Responses to Emotion

Why this matters in professional caregiving

Clients may express sadness, fear, loneliness, frustration, embarrassment, anger, grief, uncertainty, homesickness or loss of independence. A calm, validating response can reduce isolation without pretending to solve the underlying problem.

Learning objectives

The learner should be able to validate feelings; offer practical support and choice; use appropriate reassurance; and recognise when additional help is needed.

Core instructional content

Validation means acknowledging that the emotion is real and understandable from the person’ s perspective. It does not mean agreeing with every interpretation or saying that a dangerous situation is safe. “I can see this is frightening” is different from “You are right that nobody can help.”

Use calm language, simple choices and practical support: a quieter space, a phone call where authorised, a familiar routine, help contacting a supervisor or an opportunity to rest. Avoid clichés such as “Everything happens for a reason,” “Be strong” or “Others have it worse.”

Anger should not be met with retaliation. Maintain a safe distance, lower stimulation, set respectful limits and seek help if there is threat or violence. Do not physically restrain unless separately trained and authorised under applicable policy.

Applying This in Practice

A client may prefer a family member, faith leader or community contact. Support the connection only with consent and according to privacy and safeguarding rules.

Case scenario

A client says, “I am frightened about tomorrow’s procedure.”

Best response: “It makes sense to feel frightened. What information or support would help? I can help you contact the appropriate professional.”

Common mistakes / professional risks

Clichés, arguing, false reassurance, overpromising, taking anger personally, disclosing personal information and acting beyond role.

Key practice points

1. Acknowledge emotion.
2. Offer choice and practical support.
3. Reassurance should be realistic.
4. Set respectful limits around unsafe behaviour.
5. Escalate serious distress.

Lesson assessment

1. **MCQ:** Validation means: A) agreeing with every belief; B) acknowledging the person’s emotion respectfully; C) diagnosing; D) promising a cure. **Answer: B. Objective: 4.**
2. **Scenario:** A client is angry and shouting. **Answer:** Stay calm, reduce stimulation, set limits and seek help if unsafe. **Objective: 6.**
3. **True/False:** “Others have it worse” is supportive. **Answer: False. Objective: 4.**
4. **What should not happen?** Promise that an uncertain outcome will definitely be fine. **Answer:** Use honest, practical reassurance. **Objective: 4.**
5. **Application:** Give one validating statement. **Answer:** “I can see this is frightening and I’m here to help you contact the right person.” **Objective: 4.**

6. **Scenario:** Client wants an authorised family call. **Answer:** Facilitate within consent and policy. **Objective: 6.**

Lesson 6. Distress, Anger and Uncertainty

Why this matters in professional caregiving

Distress can escalate when a person feels unheard, rushed, trapped, ashamed or overwhelmed. Caregivers need a safe de-escalation approach and must know when ordinary support is no longer enough.

Learning objectives

The learner should be able to identify triggers; reduce unnecessary stimulation; use calm communication; protect safety; and escalate threats or severe distress.

Core instructional content

First check immediate safety. Keep your voice calm, use short sentences, allow space and avoid crowding. Offer a choice where possible. Reduce noise, unnecessary observers and avoid arguing about facts when the person is overwhelmed. Do not make sudden movements or block an exit unless required for immediate safety and consistent with policy.

If there is a threat toward others, inability to maintain safety, severe confusion, extreme agitation or acute deterioration, seek urgent help. The caregiver should not conduct a psychiatric risk assessment, diagnose a crisis or manage it alone.

Afterward, record objective facts: trigger if known, words used, actions, risk, support, people informed and follow-up. Do not write “bad behaviour” without description.

Applying This in Practice

In a busy Nigerian household, visitors, noise, heat or language mismatch may contribute to distress. Consider practical triggers without blaming family or culture.

Case scenario

A client becomes highly distressed when several relatives speak at once.

Best response: Reduce stimulation, use one calm speaker, offer space and seek help if safety cannot be maintained.

Common mistakes / professional risks

Crowding, shouting, arguing, threatening, touching without consent, making promises, restraining without authority and delaying urgent help.

Key practice points

1. Safety comes first.
2. Reduce stimulation.
3. Use one calm communicator.
4. Do not diagnose or manage severe crisis alone.
5. Document facts and escalation.

Lesson assessment

1. **Scenario:** Several people are arguing around a distressed client. **Answer:** Reduce stimulation and use one calm communicator. **Objective: 6.**
2. **MCQ:** A threat toward others requires: A) debate; B) urgent safety and escalation; C) secrecy; D) punishment. **Answer: B. Objective: 9.**
3. **True/False:** Physical restraint is an ordinary caregiver de-escalation tool. **Answer: False. Objective: 6.**
4. **What should not happen?** Crowd the client to force calm. **Answer:** Allow space and seek trained help. **Objective: 6.**
5. **Application:** Name three facts to document. **Answer:** Trigger, words/actions, risk, response, notifications or outcome. **Objective: 10.**
6. **Scenario:** The caregiver cannot maintain safety. **Answer:** Activate urgent support and do not manage alone. **Objective: 9.**

Module 3 — Loneliness, Connection and Meaningful Activity

Lesson 7. Loneliness and Social Isolation

Why this matters in professional caregiving

Loneliness is a subjective feeling of insufficient connection; social isolation concerns limited contact or participation. A person may have many contacts and still feel lonely. Reduced mobility, relocation, retirement, illness, disability, bereavement, family separation and stigma can increase risk.

Learning objectives

The learner should be able to recognise loneliness and isolation; explore preferred connection; offer safe opportunities; and avoid becoming the client's substitute family.

Core instructional content

Ask what connection means to the person. One client may prefer a daily phone call; another may value a quiet conversation, a faith service, a neighbour, music or a community group. Offer options rather than assuming that group activities are helpful.

Support authorised family contact, hobbies, community engagement and culturally meaningful activity. Consider transport, mobility, cost, digital access, hearing, language and safety. Do not force socialisation or share personal contact details outside policy.

Caregiver warmth is valuable, but role clarity protects both parties. A caregiver should not become the client's primary friend, borrow money, accept inappropriate gifts or provide personal availability around the clock.

Applying This in Practice

In Nigeria, diaspora families may connect by phone or video; rural communities may provide local social contact; urban clients may experience isolation despite population density. Ask what the client actually wants.

Case scenario

A client says, "You are the only person I have."

Best response: Acknowledge the importance of the relationship, offer to explore authorised connections and maintain professional boundaries. Document if the statement signals serious isolation or risk.

Common mistakes / professional risks

Forcing activities, promising friendship, sharing personal numbers, excluding family without reason, assuming a faith group is desired and ignoring access barriers.

Key practice points

1. Ask what connection means to the person.
2. Offer safe, meaningful choices.
3. Do not force social contact.
4. Caregiver warmth does not remove boundaries.
5. Escalate serious isolation or risk.

Lesson assessment

1. **MCQ:** Loneliness is best understood as: A) always being alone; B) a person's felt lack of meaningful connection; C) a diagnosis; D) bad character. **Answer: B. Objective: 5.**
2. **Scenario:** Client wants a family video call. **Answer:** Facilitate if authorised and consented. **Objective: 6.**
3. **True/False:** Every lonely person wants group activities. **Answer: False. Objective: 6.**
4. **What should not happen?** Give a personal number and promise constant availability. **Answer:** Maintain boundaries. **Objective: 8.**
5. **Application:** Name four connection options. **Answer:** Conversation, family contact, hobbies, community, faith, music or safe activity. **Objective: 6.**
6. **Scenario:** Client has no preferred contacts and is increasingly hopeless. **Answer:** Listen, document and escalate. **Objective: 5 and 9.**

Lesson 8. Meaningful Activity

Why this matters in professional caregiving

Meaningful activity supports identity, routine, confidence, connection and pleasure. It should be adult, individualised and achievable rather than a generic distraction.

Learning objectives

The learner should be able to match person, interest, ability, choice and safe participation; adapt activities; and avoid infantilising or coercive activity.

Core instructional content

Use the sequence **PERSON → INTEREST → ABILITY → CHOICE → SAFE PARTICIPATION**. Ask about past roles, hobbies, music, reading, gardening, cooking participation, religious or cultural practices, exercise within the care plan, reminiscence, creativity and social activities.

Adapt the activity to energy, vision, hearing, mobility, cognition and mood. A person who cannot complete a former task may still choose ingredients, listen to music, direct a process or share knowledge. Offer an adult role and do not equate disability with childlike preferences.

Activity is not a treatment for depression. If a person loses interest persistently or becomes unable to participate, document and report the change.

Applying This in Practice

Use familiar local music, stories, foods, crafts, gardening or faith routines where the person wants them. Avoid imposing an activity because it is convenient for staff.

Case scenario

A former tailor refuses generic games but enjoys discussing fabrics and designs.

Best response: Build activity around tailoring interests, perhaps sorting fabrics or discussing designs, with choice and safe participation.

Common mistakes / professional risks

Childish activities, forced participation, unsafe exercise, treating activity as cure, ignoring fatigue and choosing staff convenience over client meaning.

Key practice points

1. Start with the person.
2. Adapt to ability and energy.
3. Offer choice.
4. Keep activity adult and meaningful.
5. Report persistent loss of interest.

Lesson assessment

1. **MCQ:** The activity sequence is: A) task → person → force; B) person → interest → ability → choice → safe participation; C) staff → schedule; D) diagnosis → activity. **Answer: B. Objective: 6.**
2. **Scenario:** Client refuses a generic game but loves music. **Answer:** Offer a music-based activity chosen by the client. **Objective: 6.**
3. **True/False:** Activities should be imposed to improve mood. **Answer: False. Objective: 6.**
4. **What should not happen?** Treat activity as a cure for depression. **Answer:** Report persistent changes for appropriate support. **Objective: 5.**
5. **Application:** Name three adaptations. **Answer:** Shorter duration, accessible materials, seated version, quieter setting or assistance. **Objective: 6.**
6. **Scenario:** Client becomes tired during activity. **Answer:** Pause, offer choice and document if new/repeated. **Objective: 5–6.**

Lesson 9. Relationships and Independence

Why this matters in professional caregiving

Care should increase participation and autonomy where safe. Over-helping can remove confidence; under-supporting can create isolation or risk. Relationships must remain professional and respectful.

Learning objectives

The learner should be able to promote independence; support authorised relationships; recognise over-involvement; and respond to requests that blur boundaries.

Core instructional content

Ask what the person can do, wants to do and needs help with. Offer prompts, preparation, adaptive support or extra time before taking over. Support the client to choose contacts and activities. Do not make decisions merely because the caregiver is faster.

A client may invite the caregiver to family events, ask for personal errands, request loans or send intimate messages. Respond kindly but clearly: “I cannot do that as part of my professional role, but I can help you identify an authorised option.” Inform a supervisor if pressure, dependency or exploitation risk develops.

Applying This in Practice

Multigenerational families may expect a caregiver to behave like a relative. Clarify the role while respecting family relationships and local custom.

Case scenario

A client asks the caregiver to attend a private family ceremony as a friend.

Best response: Decline respectfully according to policy, explain the professional boundary and inform a supervisor if repeated pressure occurs.

Common mistakes / professional risks

Taking over tasks, accepting money, secret relationships, personal errands, dual relationships, favouritism and sharing personal problems.

Key practice points

1. Support, do not substitute.
2. Preserve choice and participation.
3. Boundaries protect both people.
4. Use clear, respectful refusals.
5. Escalate pressure or exploitation risk.

Lesson assessment

1. **Scenario:** Client can complete a task with prompts but caregiver is faster. **Answer:** Support the client and allow time. **Objective: 6.**
2. **MCQ:** A professional boundary protects: A) only the caregiver; B) both client and caregiver; C) no one; D) family convenience. **Answer: B. Objective: 8.**
3. **True/False:** Accepting regular loans from a client is appropriate if they insist. **Answer: False. Objective: 8.**
4. **What should not happen?** Become the client's private friend outside policy. **Answer:** Maintain role clarity. **Objective: 8.**
5. **Application:** Give one boundary statement. **Answer:** "I cannot do that in my professional role, but I can help find an authorised option." **Objective: 8.**
6. **Scenario:** Client is increasingly dependent on one caregiver. **Answer:** Discuss with supervisor and strengthen broader support. **Objective: 6 and 8.**

Module 4 — Grief, Adjustment and Mental-Health Awareness

Lesson 10. Loss, Grief and Bereavement

Why this matters in professional caregiving

Grief may follow death, loss of independence, mobility, employment, relationships, identity or familiar surroundings. It varies between people and may change over time. The caregiver should listen and acknowledge without rushing or diagnosing.

Learning objectives

The learner should be able to respond to grief with presence; respect cultural and faith practices; recognise severe or unsafe distress; and escalate appropriately.

Core instructional content

There is no single correct timetable or emotional pattern for grief. Some people speak; others prefer quiet. Some want family, faith or community support; others want privacy. Avoid clichés such as “move on,” “be strong” or “it was meant to be.” Say: “I am sorry this is painful. I can listen, and we can seek the support you want.”

Grief can affect sleep, appetite, activity and concentration. A caregiver observes change and reports concerns, particularly when distress is severe, prolonged, worsening, unsafe or

associated with hopelessness, self-harm or inability to maintain basic safety. Do not diagnose complicated grief.

Applying This in Practice

Respect mourning practices without assuming every family follows the same practice. Ask what the person wants and follow consent, safeguarding and organisational policy.

Case scenario

A client grieving a sibling has stopped eating and says life feels pointless.

Best response: Listen calmly, take the statement seriously, assess immediate safety only as trained through the escalation procedure, remain with the person where safe and seek urgent professional help if risk is present.

Common mistakes / professional risks

Rushing grief, empty clichés, imposing faith, promising secrecy, ignoring appetite or safety changes and attempting therapy.

Key practice points

1. Grief varies.
2. Presence is often more useful than advice.
3. Respect culture and choice.
4. Notice unsafe or worsening distress.
5. Escalate; do not diagnose.

Lesson assessment

1. **MCQ:** A supportive grief response is: A) “Move on” ; B) “I’m sorry; would you like to tell me more?” ; C) “Do not cry” ; D) ignore. **Answer: B. Objective: 7.**
2. **Scenario:** Grief is accompanied by hopeless statements. **Answer:** Take seriously and escalate through the safety pathway. **Objective: 9.**
3. **True/False:** Everyone follows the same grief timetable. **Answer: False. Objective: 7.**
4. **What should not happen?** Promise to keep a serious safety statement secret. **Answer:** Explain professional limits and seek help. **Objective: 9.**
5. **Application:** Name three losses other than bereavement. **Answer:** Independence, mobility, work, relationship, home or identity. **Objective: 7.**

6. **Scenario:** Client prefers privacy. **Answer:** Respect it while maintaining required safety observation. **Objective: 7 and 9.**

Lesson 11. Adjustment to Care and Change

Why this matters in professional caregiving

Starting home care, moving into residential care, receiving a diagnosis, becoming dependent, returning from hospital or changing living arrangements can challenge identity and control.

Learning objectives

The learner should be able to support predictable routines, continuity, choice, familiar practices and gradual adjustment.

Core instructional content

Adjustment is supported when people understand what will happen, retain meaningful choices and meet consistent caregivers. Introduce yourself, explain routines, identify preferences and avoid changing everything at once. Invite the person to decide what can remain familiar.

A person may express anger, homesickness, fear or refusal. Do not treat this automatically as non-compliance. Explore what has changed, what is missed and what control can be restored. Report persistent withdrawal, inability to cope or major change.

Applying This in Practice

In home care, the presence of a worker may feel intrusive. In residential care, family separation or unfamiliar routines may be painful. Respect language, faith, food, music, clothing and visiting preferences.

Case scenario

A client newly discharged from hospital refuses the home-care worker's help.

Best response: Introduce yourself, explain the role, offer choices, ask what would make support acceptable and report ongoing refusal or distress.

Common mistakes / professional risks

Rushing familiarity, ignoring loss of control, changing routines without explanation, forcing participation and taking refusal personally.

Key practice points

1. Predictability reduces stress.
2. Preserve control where possible.
3. Use familiar routines.
4. Treat refusal as information.
5. Escalate persistent or unsafe adjustment difficulty.

Lesson assessment

1. **Scenario:** New care arrangement causes homesickness. **Answer:** Listen, preserve familiar routine and support authorised connection. **Objective: 7.**
2. **MCQ:** Adjustment is helped by: A) sudden changes; B) predictability, continuity and choice; C) threats; D) secrecy. **Answer: B. Objective: 7.**
3. **True/False:** Refusal always means lack of cooperation. **Answer: False. Objective: 5 and 7.**
4. **What should not happen?** Take refusal personally and argue. **Answer:** Explore, offer choice and report. **Objective: 4 and 7.**
5. **Application:** Name three continuity supports. **Answer:** Familiar routine, consistent worker, preferred language, personal objects or chosen contacts. **Objective: 7.**
6. **Scenario:** Client becomes increasingly unable to cope. **Answer:** Document and escalate. **Objective: 5 and 10.**

Lesson 12. Anxiety, Low Mood and Meaningful Change

Why this matters in professional caregiving

Caregivers may observe persistent sadness, withdrawal, loss of interest, worry, restlessness, sleep or appetite changes, hopeless statements, reduced self-care, unusual irritability, confusion or marked change from baseline. These signs can have physical, medication, environmental or mental-health causes.

Learning objectives

The learner should be able to recognise possible patterns; distinguish a bad day from significant change; provide support within role; and report without diagnosis.

Core instructional content

Anxiety awareness includes persistent worry, reassurance seeking, avoidance, restlessness, difficulty settling, physical distress and sleep disruption. Support may include calm

communication, predictable routine, reducing stimulation, identifying triggers and following the care plan.

Low mood awareness includes persistent sadness, withdrawal, loss of interest, reduced participation, hopeless statements, sleep or appetite changes and reduced self-care. One bad day is not the same as persistent or significant change. Nevertheless, hopeless or unsafe statements require serious attention regardless of duration.

Use the sequence **NOTICE → RECORD → SUPPORT WITHIN SCOPE → REPORT / ESCALATE**. Do not say “the client is depressed” or “has anxiety” based solely on observation. Also consider sudden confusion or acute behavioural change as a possible urgent health concern requiring prompt review.

Applying This in Practice

Compare with the person’s usual behaviour and ask whether the change began after illness, bereavement, relocation, medication change or conflict. Report rather than speculate.

Case scenario

A normally sociable client has withdrawn for two weeks, stopped attending activities and says, “Nothing matters.”

Best response: Listen, take the statement seriously, document the change and escalate promptly through the appropriate safety pathway.

Common mistakes / professional risks

Diagnosing, minimising, giving medication advice, treating activity as a cure, ignoring confusion and failing to escalate hopelessness.

Key practice points

1. Patterns and baseline matter.
2. Do not diagnose.
3. Hopelessness deserves serious attention.
4. Support calmly within scope.
5. Escalate significant or unsafe change.

Lesson assessment

1. **MCQ:** The correct caregiver response to possible depression signs is: A) diagnose; B) notice, record, support and report; C) prescribe; D) ignore. **Answer: B. Objective: 5.**

2. **Scenario:** Client is withdrawn for two weeks and hopeless. **Answer:** Take seriously and escalate promptly. **Objective: 9.**
3. **True/False:** One sad afternoon proves depression. **Answer: False. Objective: 5.**
4. **What should not happen?** Advise the client to stop prescribed medication. **Answer:** Medication decisions require authorised professionals. **Objective: 5.**
5. **Application:** Name four possible signs of concern. **Answer:** Persistent sadness, withdrawal, loss of interest, worry, sleep/appetite change, hopelessness or reduced self-care. **Objective: 5.**
6. **Scenario:** Sudden confusion appears. **Answer:** Report promptly for appropriate assessment. **Objective: 5 and 10.**
7. **Documentation:** Record baseline, change, words, duration, support and notifications. **Answer:** Correct. **Objective: 10.**

Module 5 — Boundaries, Confidentiality and Trauma-Informed Care

Lesson 13. Professional Boundaries

Why this matters in professional caregiving

Boundaries protect trust, safety, fairness and dignity. Emotional warmth is compatible with professional distance. Over-involvement can create dependency, favouritism, exploitation risk and distress when the worker is unavailable.

Learning objectives

The learner should be able to recognise boundary crossings; respond to gifts, money, requests and messages; and seek supervision.

Core instructional content

Boundary risks include borrowing or lending money, accepting significant gifts, private meetings, secret relationships, sexualised communication, personal errands, social-media contact outside policy, sharing personal crises and promising special treatment. A client's consent does not remove organisational or professional limits.

Use a respectful script: "I appreciate that you trust me, but I cannot accept this or do that outside my role. I will discuss an appropriate option with my supervisor." Record and report repeated pressure, coercion, exploitation, harassment or conflicts of interest.

If the caregiver notices emotional over-involvement—thinking about one client constantly, favouring them, hiding contact or feeling responsible for solving all problems—seek supervision early.

Applying This in Practice

Family expectations may blur roles. Clarify that the caregiver is part of the care team, not a private family member, while remaining respectful of household norms.

Case scenario

A client offers the caregiver money to visit on days off and asks them to keep it secret.

Best response: Decline, explain the boundary, do not keep the secret, document and inform the supervisor.

Common mistakes / professional risks

Secret contact, accepting money, special treatment, personal dependency, romantic involvement and failure to report.

Key practice points

1. Boundaries protect both parties.
2. Consent does not authorise prohibited conduct.
3. Use clear, kind limits.
4. Seek supervision when over-involved.
5. Report exploitation or harassment concerns.

Lesson assessment

1. **MCQ:** A professional boundary is: A) rejection; B) a safe limit that protects the relationship; C) secrecy; D) friendship. **Answer: B. Objective: 8.**
2. **Scenario:** Client offers money for secret visits. **Answer:** Decline, report and document. **Objective: 8–10.**
3. **True/False:** A client's permission makes any personal relationship acceptable. **Answer: False. Objective: 8.**
4. **What should not happen?** Hide private messages from the supervisor. **Answer:** Follow digital and reporting policy. **Objective: 8, 10.**
5. **Application:** Give one boundary statement. **Answer:** “I cannot accept money or meet privately, but I can help through the authorised service.” **Objective: 8.**

6. **Scenario:** Caregiver feels responsible for solving all problems. **Answer:** Seek supervision and maintain role. **Objective: 8.**
7. **Documentation:** Record boundary incident factually. **Answer:** Correct. **Objective: 10.**

Lesson 14. Confidentiality and Digital Conduct

Why this matters in professional caregiving

Sensitive emotional information can harm a client if disclosed casually. Confidentiality supports trust but is not an absolute promise when safety, safeguarding or legal/professional obligations require information sharing.

Learning objectives

The learner should be able to protect sensitive information; explain limits; use approved communication channels; and avoid inappropriate digital contact.

Core instructional content

Share information only with people who need it for care, safeguarding or authorised follow-up. Use approved records and systems. Do not post client stories, photographs, voice notes or emotional disclosures on personal social media. Avoid discussing clients in public areas, transport, places of worship or family groups.

When a client asks for secrecy about serious risk, respond honestly: “I will respect your privacy, but I cannot keep information secret if someone may be unsafe. I will share only what is needed with the right person.” Do not promise absolute confidentiality.

Check consent before supporting family or faith contact. A relative’s request for information does not automatically authorise disclosure. Follow policy and lawful instructions.

Applying This in Practice

Personal phones are common in home care, but professional communication should use approved channels. If an urgent disclosure arrives by personal message, protect safety first, then transfer the information to the approved system and notify the appropriate person.

Case scenario

A family member asks what the client said about feeling hopeless.

Best response: Follow consent and privacy policy, disclose only through authorised channels and escalate any safety concern directly.

Common mistakes / professional risks

Screenshots, group chats, gossip, personal social media, photographing records, sharing more than necessary and promising secrecy.

Key practice points

1. Privacy is part of psychological safety.
2. Share on a need-to-know basis.
3. Use approved systems.
4. Explain limits honestly.
5. Safety concerns require escalation.

Lesson assessment

1. **MCQ:** Confidentiality means: A) never sharing anything; B) protecting information and sharing appropriately for care and safety; C) gossip; D) public disclosure. **Answer: B. Objective: 8 and 10.**
2. **Scenario:** Family requests private emotional information. **Answer:** Check consent/policy and do not disclose automatically. **Objective: 8.**
3. **True/False:** Personal social media is acceptable for client disclosures if the account is private. **Answer: False. Objective: 8.**
4. **What should not happen?** Promise absolute secrecy about suicide concern. **Answer:** Explain limits and escalate. **Objective: 9.**
5. **Application:** Name three unsafe digital practices. **Answer:** Posting photos, personal group chats, screenshots, gossip or unapproved messaging. **Objective: 8.**
6. **Scenario:** Urgent risk arrives by personal message. **Answer:** Protect safety, notify urgently and transfer to approved documentation. **Objective: 9–10.**
7. **Documentation:** Share only relevant facts with authorised recipients. **Answer:** Correct. **Objective: 10.**

Lesson 15. Trauma-Informed, Stigma-Aware Practice

Why this matters in professional caregiving

Some people receiving care have experienced violence, neglect, discrimination, displacement, loss, medical trauma or other overwhelming events. Trauma-informed awareness does not make the caregiver a trauma therapist. It means recognising that safety, control and trust matter.

Learning objectives

The learner should be able to promote safety, choice, collaboration and predictability; avoid unnecessary re-traumatisation; and respond without stigma.

Core instructional content

Use a “what may have happened?” rather than “what is wrong with this person?” mindset. Explain before touching, offer choices, avoid unnecessary exposure, respect privacy and do not demand a detailed trauma story. Be aware that a smell, tone, position, room, person or procedure may trigger distress.

Do not promise that disclosure will remain secret, investigate allegations beyond role or pressure the person to confront memories. Listen, validate, protect safety, follow safeguarding procedure and refer onward.

Trauma-informed care also includes staff self-awareness. Strong reactions, avoidance or rescuing can affect care. Use supervision and follow staff wellbeing procedures.

Applying This in Practice

Language, faith, migration, conflict, poverty and discrimination may shape experiences in Nigerian communities. Do not assume a person’s story or use culture as an explanation for distress without listening.

Case scenario

A client panics when a caregiver stands behind them and reaches suddenly toward their shoulder.

Best response: Step back, explain, offer choice and ask what would feel safer. Report recurring triggers if relevant to the care plan.

Common mistakes / professional risks

Unexpected touch, demanding disclosure, blaming, stereotyping, investigating, rescuing and ignoring worker supervision needs.

Key practice points

1. Promote safety, choice and control.
2. Explain before touching.
3. Do not demand a trauma history.
4. Do not investigate beyond role.
5. Use supervision and safeguarding pathways.

Lesson assessment

1. **MCQ:** Trauma-informed care emphasises: A) control by staff; B) safety, choice, collaboration and predictability; C) forced disclosure; D) diagnosis. **Answer: B. Objective: 8.**
2. **Scenario:** Client panics when approached from behind. **Answer:** Step back, explain and offer safer choices. **Objective: 8.**
3. **True/False:** The caregiver should ask for every trauma detail. **Answer: False. Objective: 15.**
4. **What should not happen?** Investigate an allegation independently. **Answer:** Listen, protect and follow safeguarding procedure. **Objective: 9.**
5. **Application:** Name three trauma-informed behaviours. **Answer:** Explain, offer choice, respect privacy, avoid surprise touch, collaborate. **Objective: 8.**
6. **Scenario:** Caregiver feels overwhelmed by disclosure. **Answer:** Maintain immediate support, escalate appropriately and seek supervision. **Objective: 15.**
7. **Scope:** Is trauma-informed awareness therapy? **Answer: No. Objective: 15.**

Module 6 — Safeguarding, Crisis Recognition and Escalation

Lesson 16. Safeguarding and Serious Concern

Why this matters in professional caregiving

Emotional distress can be linked to abuse, neglect, coercive control, exploitation, discrimination or unsafe living conditions. Caregivers need to recognise concern, preserve safety and report through the correct pathway.

Learning objectives

The learner should be able to recognise safeguarding indicators; respond to disclosure; avoid promising secrecy; and escalate.

Core instructional content

Possible indicators include fear of a particular person, unexplained injuries, controlling access to money or communication, sudden withdrawal, threats, humiliation, sexualised behaviour from others, missing medication or neglect of basic needs. No single sign proves

abuse. The caregiver should listen, record words accurately, avoid leading questions, protect immediate safety and report.

If a client discloses abuse, remain calm and say, “I’m glad you told me. I will take this seriously.” Do not investigate, confront the alleged perpetrator, promise secrecy or require the person to repeat the story to multiple people. Follow safeguarding policy and emergency procedure.

Applying This in Practice

Family closeness does not eliminate safeguarding risk. In multigenerational homes, respect roles while remaining alert to coercion, financial control and violence. Use authorised interpreters and private conversation where safe.

Case scenario

A client whispers that a relative takes their money and threatens them if they speak.

Best response: Ensure immediate safety, listen without investigation, document exact words and escalate through safeguarding urgently.

Common mistakes / professional risks

Confronting family, investigating, doubting, promising secrecy, sharing gossip and delaying because the alleged person is a relative.

Key practice points

1. Take concerns seriously.
2. Protect immediate safety.
3. Record words and facts.
4. Do not investigate or confront.
5. Escalate through safeguarding.

Lesson assessment

1. **MCQ:** A safeguarding disclosure requires: A) secrecy; B) calm listening, safety and reporting; C) confrontation; D) investigation by caregiver. **Answer: B. Objective: 9.**
2. **Scenario:** Client reports financial threats. **Answer:** Protect, document exact words and escalate. **Objective: 9–10.**
3. **True/False:** Family status proves there is no abuse risk. **Answer: False. Objective: 9.**
4. **What should not happen?** Confront the alleged perpetrator alone. **Answer:** Follow safeguarding procedure. **Objective: 9.**

5. **Application:** Name three safeguarding indicators. **Answer:** Fear, coercion, unexplained injury, control of money, neglect, humiliation or sudden withdrawal. **Objective: 9.**
6. **Scenario:** Client asks for secrecy. **Answer:** Explain limits and report safely. **Objective: 9.**
7. **Documentation:** Record words, time, context, immediate safety and notifications. **Answer:** Correct. **Objective: 10.**

Lesson 17. Self-Harm, Suicide and Crisis Awareness

Why this matters in professional caregiving

Statements such as “I don’ t want to be here anymore,” “Everyone would be better without me” or “I wish I could die” must be taken seriously. Caregivers should not provide methods or detailed information. They should respond calmly, stay with the person where immediate safety requires it and where safe to do so, and seek urgent help.

Learning objectives

The learner should be able to recognise concerning statements; respond without dismissal or shame; activate urgent help; avoid secrecy; and document objectively.

Core instructional content

Do not dismiss, shame, challenge aggressively or say that the person is merely seeking attention. Do not promise confidentiality beyond professional limits. Do not attempt to manage serious risk alone or conduct an independent psychiatric risk assessment.

A safe caregiver response is to listen calmly, acknowledge the concern, ask only the safety questions required by the approved local procedure or training, stay with the person where safe, reduce immediate danger only within policy, contact emergency or professional support and notify the designated supervisor/safeguarding lead. If there is immediate danger, urgent emergency assistance takes priority over routine approval.

Document the person’ s words as accurately as possible, time, context, observed behaviour, action taken, people contacted and outcome. Do not record methods or unnecessary detail.

Applying This in Practice

NIC must insert current Nigerian emergency contacts, local referral routes and employer procedures before publication. These may differ by setting. The online course should not create a universal emergency number or imply that caregivers can provide crisis treatment.

Case scenario

A client says, “Everyone would be better without me,” then asks the caregiver to keep it secret.

Best response: Take it seriously, remain calm, explain that safety information cannot be kept secret, stay with the person where safe and activate urgent support according to the local procedure.

Common mistakes / professional risks

Dismissal, shame, secrecy promises, leaving the person alone when unsafe, searching for methods, confronting aggressively and delaying help.

Key practice points

1. Take concerning statements seriously.
2. Stay calm and protect safety.
3. Do not promise secrecy.
4. Seek urgent professional/emergency help.
5. Do not manage serious risk alone.

Lesson assessment

1. **MCQ:** “I wish I could die” should be: A) dismissed; B) taken seriously and escalated; C) kept secret; D) laughed off. **Answer: B. Objective: 9.**
2. **Scenario:** Client asks for secrecy about hopelessness. **Answer:** Explain limits and activate safety support. **Objective: 9.**
3. **True/False:** A caregiver should manage serious suicide risk alone. **Answer: False. Objective: 9.**
4. **What should not happen?** Leave the person alone where immediate danger is present and no safe handover exists. **Answer:** Stay where safe and obtain urgent help. **Objective: 9.**
5. **Application:** Name four actions. **Answer:** Listen calmly, take seriously, stay where safe, seek urgent help, notify and document. **Objective: 9–10.**
6. **Scenario:** Immediate danger is present. **Answer:** Emergency response takes priority over routine approval. **Objective: 9.**
7. **Documentation:** Record exact words and actions, not methods. **Answer:** Correct. **Objective: 10.**

Lesson 18. Documentation, Handover and Integrated Judgement

Why this matters in professional caregiving

A clear record supports continuity and enables appropriate professional assessment. Integrated judgement is required when loneliness, grief, boundary pressure, family conflict, safeguarding and crisis concerns overlap.

Learning objectives

The learner should be able to document objectively; hand over relevant information; apply CONNECT; and prioritise safety and scope.

Core instructional content

Record date, time, setting, observed behaviour, exact words where important, emotional presentation, relevant trigger, support offered, client response, risk, people informed and follow-up. Avoid diagnoses and labels. “Client said, ‘I don’ t want to be here anymore,’ and moved away from others” is more useful than “client is suicidal,” unless that is an authorised professional assessment.

Handover should say what changed, what helped, what remains concerning, what must be avoided, who has been contacted and what happens next. Urgent escalation is not replaced by a form.

Apply CONNECT: connect respectfully; open the conversation; notice and listen; name and acknowledge; enable practical support; connect onward; track boundaries and facts. The urgent override applies whenever safety cannot be maintained.

Applying This in Practice

Use NIC or employer-approved Psychosocial Observation Checklist, Emotional Distress Escalation Guide and Objective Documentation Template. Protect records and share only with authorised recipients.

Case scenario

A caregiver has become the only person a lonely client trusts. The client sends private messages expressing hopelessness and asks the caregiver not to tell anyone.

Best response: Treat hopelessness seriously, protect safety, explain confidentiality limits, notify the designated professional/safeguarding pathway, document objectively and seek supervision regarding dependency and boundaries.

Common mistakes / professional risks

Vague records, delayed escalation, personal messaging, over-involvement, diagnosis, secrecy promises and treating routine documentation as a substitute for urgent help.

Key practice points

1. Record facts and exact words.
2. Document risk, response and notifications.
3. Handover what to do and avoid.
4. Use CONNECT and the urgent override.
5. Maintain scope and confidentiality.

Lesson assessment

1. **Integrated scenario:** Client expresses hopelessness by private message. **Answer:** Protect safety, escalate urgently, document and address boundary risk. **Objective: 8–10.**
2. **MCQ:** Objective documentation is: A) “attention seeker” ; B) exact words, observations, actions and notifications; C) diagnosis; D) gossip. **Answer: B. Objective: 10.**
3. **True/False:** A form replaces urgent escalation. **Answer: False. Objective: 9–10.**
4. **What should not happen?** Delete personal messages without transferring relevant safety information. **Answer:** Preserve and report according to policy. **Objective: 8–10.**
5. **Application:** List CONNECT steps. **Answer:** Connect, Open, Notice, Name, Enable, Connect onward, Track. **Objective: 9–10.**
6. **Scenario:** Client asks for secrecy about danger. **Answer:** Explain limits and escalate. **Objective: 9.**
7. **Scope:** Can the caregiver provide psychotherapy? **Answer: No. Objective: 3 and 8.**

13. Lesson assessments

Each lesson contains six or seven assessment items. NIC should provide immediate feedback explaining the communication, safety, scope and professional reasoning. Items may be randomised while preserving the learning objective and safety standard.

14. Module assessments

Module 1 assessment

1. **Scenario:** Client withdraws and is labelled difficult. **Answer:** Observe, listen and report; do not label. **LO: 1, 5.**
2. **MCQ:** Psychological safety includes predictability and privacy. **Answer:** Correct. **LO: 2.**
3. **What should not happen?** Humiliate a client publicly. **Answer:** Protect dignity. **LO: 2.**

4. **Scenario:** Family imposes a cultural assumption. **Answer:** Ask the client and follow consent and safety policy. **LO:** 3.
5. **True/False:** A caregiver may diagnose from observation. **Answer:** False. **LO:** 5.
6. **Documentation:** Record observable change, context and escalation. **Answer:** Correct. **LO:** 10.

Module 2 assessment

1. **MCQ:** Active listening includes silence and reflection. **Answer:** Correct. **LO:** 3.
2. **Scenario:** Client says they feel invisible. **Answer:** Validate and invite more. **LO:** 3–4.
3. **What should not happen?** Say “others have it worse.” **Answer:** Avoid minimisation. **LO:** 4.
4. **Scenario:** Distress escalates amid crowding. **Answer:** Reduce stimulation and seek help if unsafe. **LO:** 6.
5. **True/False:** Supportive communication is psychotherapy. **Answer:** False. **LO:** 3.
6. **Documentation:** Record exact words, action and outcome. **Answer:** Correct. **LO:** 10.

Module 3 assessment

1. **Scenario:** Client feels isolated. **Answer:** Explore preferred connection and offer safe options. **LO:** 5–6.
2. **MCQ:** Activities should begin with the person and their interests. **Answer:** Correct. **LO:** 6.
3. **What should not happen?** Force group activity. **Answer:** Preserve choice. **LO:** 6.
4. **Scenario:** Client wants a caregiver friendship. **Answer:** Acknowledge, maintain boundary and strengthen authorised support. **LO:** 8.
5. **True/False:** Caregiver availability should be unlimited. **Answer:** False. **LO:** 8.
6. **Application:** Use person → interest → ability → choice → safe participation. **Answer:** Correct. **LO:** 6.

Module 4 assessment

1. **Scenario:** Grieving client says life is pointless. **Answer:** Listen, take seriously and escalate safety concern. **LO:** 7, 9.
2. **MCQ:** Adjustment is supported by continuity and choice. **Answer:** Correct. **LO:** 7.
3. **What should not happen?** Tell the client to move on. **Answer:** Do not rush grief. **LO:** 7.
4. **Scenario:** Two-week withdrawal and hopelessness. **Answer:** Report promptly without diagnosing. **LO:** 5, 9.

5. **True/False:** One bad day proves depression. **Answer:** False. **LO:** 5.
6. **Application:** Record baseline, change, duration and response. **Answer:** Correct. **LO:** 10.
7. **Scenario:** Sudden confusion. **Answer:** Promptly escalate for appropriate assessment. **LO:** 5, 10.

Module 5 assessment

1. **Scenario:** Client offers money for secrecy. **Answer:** Decline, document and report. **LO:** 8, 10.
2. **MCQ:** Confidentiality is not an absolute promise when safety requires escalation. **Answer:** Correct. **LO:** 8–9.
3. **What should not happen?** Post client content on personal social media. **Answer:** Use approved systems. **LO:** 8.
4. **Scenario:** Client panics after unexpected touch. **Answer:** Step back, explain and offer choice. **LO:** 8.
5. **True/False:** Trauma-informed care requires therapy. **Answer:** False. **LO:** 15.
6. **Application:** Name three boundary risks. **Answer:** Money, secret contact, gifts, private meetings, special treatment or sexualised communication. **LO:** 8.
7. **Documentation:** Record relevant facts securely. **Answer:** Correct. **LO:** 10.

Module 6 assessment

1. **Scenario:** Client reports threats by a relative. **Answer:** Protect, document and escalate safeguarding. **LO:** 9–10.
2. **MCQ:** “I wish I could die” requires serious attention. **Answer:** Correct. **LO:** 9.
3. **What should not happen?** Promise secrecy about immediate danger. **Answer:** Explain limits and seek urgent help. **LO:** 9.
4. **Scenario:** Threat toward others. **Answer:** Protect safety and activate urgent support. **LO:** 9.
5. **True/False:** A completed form replaces urgent help. **Answer:** False. **LO:** 10.
6. **Application:** Record exact words, context, action and notifications. **Answer:** Correct. **LO:** 10.
7. **Integrated scenario:** Hopeless message plus boundary dependency. **Answer:** Urgent safety escalation, objective documentation and supervision. **LO:** 8–10.

15. Final assessment blueprint

The final assessment contains **35 questions**. The recommended pass mark is **80% (28/35)**. **28 questions are scenario/application based**, exceeding the required minimum of 23.

Content area	Questions	Number	Scenario/application
Emotional wellbeing and psychosocial needs	1–5	5	3
Communication and active listening	6–10	5	4
Loneliness, activity, grief and adjustment	11–15	5	4
Anxiety, low mood and awareness	16–20	5	4
Boundaries, confidentiality and stigma	21–25	5	4
Safeguarding, self-harm and crisis	26–30	5	5
Integrated judgement	31–35	5	4
Total		35	28

16. Final assessment questions

1. **MCQ:** Psychosocial support is: A) psychotherapy; B) practical, relational and social support within role; C) diagnosis; D) medication management. **Answer: B. LO: 1.**
2. **Scenario:** A client withdraws from conversation and is called difficult. **Answer:** Observe, listen, document and report meaningful change; do not label. **LO: 1, 5, 10.**
3. **MCQ:** Emotional wellbeing includes: A) only happiness; B) dignity, belonging, identity and autonomy; C) diagnosis; D) compliance. **Answer: B. LO: 1.**
4. **Scenario:** A relative dismisses sadness as attention seeking. **Answer:** Continue respectful support, observe the client and report concern. **LO: 5.**
5. **What should not happen?** Diagnose depression from withdrawal alone. **Answer:** Awareness is not diagnosis. **LO: 5.**
6. **MCQ:** Active listening includes: A) interrupting; B) reflection, silence and clarification; C) advice only; D) changing topic. **Answer: B. LO: 3.**

7. **Scenario:** Client says, “I feel invisible.” **Answer:** “That sounds lonely. Would you like to tell me more?” **LO:** 3–4.
8. **Scenario:** Client cries after describing fear. **Answer:** Remain present, allow silence and acknowledge emotion. **LO:** 3–4.
9. **What should not happen?** Say “others have it worse.” **Answer:** Avoid minimisation. **LO:** 4.
10. **Scenario:** Client says they feel unsafe. **Answer:** Clarify enough to protect safety and escalate. **LO:** 3, 9.
11. **Scenario:** Client is lonely but dislikes groups. **Answer:** Explore preferred one-to-one, family, faith, hobby or community connection. **LO:** 5–6.
12. **MCQ:** Meaningful activity begins with: A) staff convenience; B) person, interest, ability, choice and safe participation; C) forced exercise; D) diagnosis. **Answer:** B. **LO:** 6.
13. **Scenario:** Former tailor rejects generic games. **Answer:** Offer tailoring-related activity chosen by the client. **LO:** 6.
14. **Scenario:** Client says caregiver is their only friend. **Answer:** Acknowledge, maintain boundaries and build authorised supports. **LO:** 6, 8.
15. **What should not happen?** Force social contact or activity. **Answer:** Preserve choice. **LO:** 5–6.
16. **Scenario:** Grieving client has stopped eating and says life is pointless. **Answer:** Listen, take seriously, protect safety and escalate. **LO:** 7, 9.
17. **MCQ:** Grief: A) follows one timetable; B) varies between people; C) should be rushed; D) is always a disorder. **Answer:** B. **LO:** 7.
18. **Scenario:** New care arrangement causes homesickness. **Answer:** Use predictable routine, choice and authorised connection. **LO:** 7.
19. **Scenario:** Client worries repeatedly about tomorrow. **Answer:** Calmly identify triggers, offer predictability and report persistent concern. **LO:** 5.
20. **What should not happen?** Tell the client they “have anxiety” based only on worry. **Answer:** Observe and report without diagnosis. **LO:** 5.
21. **Scenario:** Client offers money for secret visits. **Answer:** Decline, document, report and maintain boundary. **LO:** 8, 10.
22. **MCQ:** Confidentiality means: A) absolute secrecy; B) appropriate protection and need-to-know sharing; C) public disclosure; D) gossip. **Answer:** B. **LO:** 8.
23. **What should not happen?** Share a client’s disclosure in a personal group chat. **Answer:** Use approved systems. **LO:** 8, 10.

24. **Scenario:** Client asks caregiver to become a private family friend. **Answer:** Respond kindly, explain role limits and support authorised connection. **LO:** 8.
25. **Scenario:** Client panics after unexpected touch. **Answer:** Step back, explain and offer control. **LO:** 15.
26. **Scenario:** Client says a relative threatens them for money. **Answer:** Protect safety, listen, document exact words and escalate safeguarding. **LO:** 9–10.
27. **MCQ:** “I wish I could die” should be: A) dismissed; B) taken seriously and escalated; C) kept secret; D) challenged aggressively. **Answer:** B. **LO:** 9.
28. **Scenario:** Client asks for secrecy about hopelessness. **Answer:** Explain limits, stay where safe and seek urgent help. **LO:** 9.
29. **What should not happen?** Manage serious suicide concern alone. **Answer:** Activate urgent professional/emergency support. **LO:** 9.
30. **Scenario:** Threat toward another person and inability to maintain safety. **Answer:** Protect safety and activate urgent support. **LO:** 9.
31. **Scenario:** A client is isolated, withdrawn and refusing all activities. **Answer:** Listen, assess change from baseline, offer choice, document and report. **LO:** 5–6, 10.
32. **Scenario:** Family demands details of a client’s private emotional disclosure. **Answer:** Follow consent and confidentiality policy; share only appropriately. **LO:** 8.
33. **Scenario:** Caregiver receives a suicidal message on a personal phone. **Answer:** Protect safety, notify urgent contacts, transfer information to approved records and seek supervision. **LO:** 8–10.
34. **What should not happen?** Delete the message and continue the shift without reporting. **Answer:** Serious concern must be escalated. **LO:** 9–10.
35. **Integrated scenario:** A lonely client says, “Everyone would be better without me,” asks the caregiver to keep it secret and has become dependent on that one worker. **Answer:** Connect calmly; listen and take the statement seriously; explain confidentiality limits; stay with the person where safe; activate urgent professional/emergency and safeguarding pathways; do not manage alone; document exact words and actions; hand over; and seek supervision for boundary dependency. **LO:** 1–10.

17. Answer key and explanations

Q	Answer	Reasoning
1	B	Caregivers provide practical and relational support, not therapy.

2	Observe, document, report	Behaviour is information; labels obscure risk.
3	B	Wellbeing includes dignity, identity, belonging and autonomy.
4	Respectful observation and escalation	Family dismissal does not remove caregiver responsibility.
5	Do not diagnose	Signs require appropriate assessment.
6	B	Listening uses presence, reflection, silence and clarification.
7	Validate and invite	This acknowledges emotion without minimising.
8	Presence and acknowledgment	Silence may be supportive.
9	Do not minimise	Comparison can increase isolation.
10	Clarify and escalate	“Unsafe” may indicate immediate risk.
11	Explore preferred connection	Social support should be individualised.
12	B	Meaningful activity begins with the person.
13	Tailoring-related choice	Activity should reflect identity and interest.
14	Boundaries plus authorised connection	Caregiver should not become substitute family.
15	Do not force	Choice and dignity matter.
16	Listen and escalate	Hopelessness with reduced intake is concerning.
17	B	Grief varies across people and cultures.
18	Predictability, choice, connection	These support adjustment.

19	Calm support and report	Persistent anxiety requires review.
20	Do not diagnose	Awareness is not clinical assessment.
21	Decline and report	Money and secrecy create boundary risk.
22	B	Confidentiality has appropriate safety limits.
23	Do not use personal chat	Protect privacy and use approved systems.
24	Kind role clarity	Warmth and boundaries coexist.
25	Step back and offer control	This is trauma-informed practice.
26	Safeguarding escalation	Threats and financial control may indicate abuse.
27	B	Death-related statements require serious attention.
28	Explain limits and seek help	Safety overrides secrecy.
29	Do not manage alone	Serious risk requires urgent support.
30	Protect and escalate	Immediate danger takes priority.
31	Integrated support	Pattern, choice, documentation and escalation matter.
32	Follow policy	Family request alone does not authorise disclosure.
33	Urgent safety plus approved reporting	Personal-message risk must not disappear.
34	Do not delete or ignore	Failure to report may leave the client unsafe.
35	CONNECT plus urgent override	The response integrates support, scope, safety and boundaries.

18. Recommended behavioural competency verification

Online learning demonstrates knowledge and scenario judgement. It does not prove counselling, psychotherapy, psychiatric crisis-management, safeguarding investigation or emergency-response competence.

Verification method	What it tests	Evidence
Listening role-play	Presence, reflection, validation and boundaries	Assessor rubric
Distress scenario	Calm communication, space, choice and escalation	Scenario score
Boundary scenario	Handling money, messages, gifts and dependency	Observation checklist
Safeguarding disclosure simulation	Listening, exact documentation and reporting	Structured exercise
Crisis simulation	Activating urgent help and safe handover	Practical sign-off
Supervisor observation	Real-world communication and professionalism	Signed verification
Reflective practice	Recognition of personal limits and supervision need	Short reflection

Critical-fail behaviours should include dismissing suicidal statements, promising secrecy about danger, leaving a person in immediate danger without safe handover, confronting an alleged abuser, diagnosing, providing therapy, secret personal relationships, accepting exploitative payments or falsifying records.

19. Completion requirements

NIC should issue the micro-credential when the learner has completed all 18 lessons, attempted all six module assessments, achieved at least 80% on the final assessment, completed the scope acknowledgement and reviewed the emergency/safeguarding limitation. NIC may allow up to three final-assessment attempts, with required review after failure.

The online credential demonstrates knowledge and scenario-based judgement in respectful psychosocial support, active listening, emotional wellbeing awareness, loneliness and meaningful activity, grief and adjustment, boundary and confidentiality practice, crisis

recognition, documentation and escalation. It does not demonstrate counselling, psychotherapy, psychiatric assessment, clinical diagnosis, suicide-risk assessment, crisis-management or emergency-response competence.

NIC should retain learner identity, enrolment, completion date, curriculum version, module and final scores, assessment attempts, scope acknowledgement, certificate ID and behavioural-verification records where applicable. Records should be handled securely.

20. Certificate wording

Certificate title

NIC CPD Micro-Credential: Mental Health, Emotional Wellbeing & Psychosocial Support for Professional Caregivers

Competency statement

This credential recognises successful completion of NIC continuing professional development in mental health awareness, emotional wellbeing and psychosocial support. The learner demonstrated knowledge-based and scenario-based understanding of person-centred emotional support, psychological safety, active listening, supportive communication, loneliness and social connection, meaningful activity, grief and adjustment, anxiety and low-mood awareness, professional boundaries, confidentiality, trauma-informed practice, safeguarding, crisis recognition, documentation and escalation within caregiver scope.

The certificate must state the 5–6 CPD learning hours, completion date, certificate identifier and curriculum version. It must also state:

This CPD micro-credential is not a psychology, psychiatry, counselling, psychotherapy, psychiatric nursing, clinical mental-health or crisis-specialist qualification. It does not authorise the holder to diagnose mental-health conditions, provide therapy, prescribe medication, conduct independent psychiatric risk assessments or manage serious mental-health emergencies independently.

21. Digital badge specification

Field	Recommended value
Badge name	NIC CPD Mental Health & Psychosocial Support

Description	Recognises professional development in respectful, person-centred emotional and psychosocial support within caregiver scope.
Competencies	Psychological safety; active listening; supportive communication; connection; meaningful activity; grief and adjustment awareness; mental-health awareness; boundaries; confidentiality; safeguarding; crisis recognition; documentation and escalation
CPD hours	5–6
Level	Professional CPD; intermediate workplace application
Evidence	18 lessons, six module assessments, final assessment at 80%, scope acknowledgement and behavioural verification where required
Verification	NIC verification URL/QR, certificate ID, completion date and curriculum version
Limitation	Not a psychology, counselling, therapy, psychiatric or crisis-management qualification
Review period	Recommend review every 24 months or sooner when guidance, law, policy or emergency procedures change

22. NIC portal course description

Mental Health, Emotional Wellbeing & Psychosocial Support for Professional Caregivers is a 5–6 hour NIC CPD micro-credential for caregivers who already understand foundational care. It develops practical skills for listening respectfully, supporting emotional wellbeing, encouraging connection and meaningful activity, recognising significant changes, maintaining boundaries and confidentiality, and escalating safeguarding or crisis concerns.

Learners practise the **CONNECT** framework: Connect respectfully, Open the conversation, Notice and listen, Name emotion, Enable practical support, Connect onward, and Track boundaries and facts. The course is not counselling or psychotherapy training and does not qualify learners to diagnose, prescribe or independently manage psychiatric emergencies.

23. Recommended downloadable job aids

A. Psychosocial Observation Checklist

Observation	Yes / No / N/A	Notes
Change from usual mood or participation		
Sadness, fear, worry, anger or grief expressed		
Withdrawal or reduced social connection		
Loss of interest or meaningful activity		
Sleep, appetite or self-care change reported/observed		
Confusion or unusual behaviour		
Hopeless or death-related statement		
Fear of a person or safeguarding concern		
Support offered and client response		
Person informed and next action		

B. Supportive Communication Guide

Use: “I notice you seem quieter today.” “That sounds difficult.” “Would you like to tell me more?” “What would help right now?” “I can help you contact the appropriate person.” Avoid: “Don’ t worry.” “Others have it worse.” “You are attention seeking.” “I know exactly how you feel.” “You must keep this secret.”

C. Professional Boundaries Checklist

Check for personal money or gifts, secret contact, private social-media connection, inappropriate messages, favouritism, personal errands, dual relationships, over-

involvement, pressure to keep secrets and unclear role expectations. Record and seek supervision when risk appears.

D. Emotional Distress Escalation Guide

Immediate emergency: immediate self-harm or suicide danger, serious threat toward others, inability to maintain safety, severe confusion, collapse or acute deterioration. Protect safety, stay where safe and activate emergency response.

Urgent professional escalation: hopeless statements, suspected abuse, severe distress, sudden extreme change, persistent inability to cope, unsafe neglect or serious safeguarding concern. Notify designated professional/safeguarding lead promptly.

Routine review: persistent withdrawal, loneliness, anxiety, low mood, adjustment difficulty, sleep/appetite change or loss of interest without immediate danger.

NIC must insert current Nigerian emergency contacts, local referral pathways, safeguarding contacts and employer procedures before release.

E. Objective Documentation Template

Record date, time, setting, baseline, observed change, exact words, relevant context, support provided, client response, immediate safety concern, person contacted, time of escalation, advice received and follow-up. Avoid diagnosis, labels, unnecessary trauma details and speculation.

24. References

- [1] World Health Organization, Psychological first aid: Guide for field workers.
- [2] World Health Organization, Guidance on community mental health services: Promoting person-centred and rights-based approaches.
- [3] World Health Organization, Mental health: strengthening our response.
- [4] World Health Organization, mhGAP Intervention Guide for mental, neurological and substance use disorders in non-specialized health settings.
- [5] World Health Organization, Preventing suicide: a resource for media professionals, update 2023.
- [6] U.S. Centers for Disease Control and Prevention, Risk and Protective Factors for Suicide.
- [7] Substance Abuse and Mental Health Services Administration, Suicide Prevention and 988 resources.
- [8] World Health Organization, Abuse of older people.
- [9] World Health Organization, Global status report on preventing violence against children 2020.
- [10] World Health Organization, Basic psychosocial skills: A guide for COVID-19 responders.

[11] National Institute of Mental Health, Older Adults and Mental Health.

[12] National Health Service, Mental health conditions and support.

Source-use statement: The cited materials provide international and evidence-informed guidance. They are not Nigerian law, NIC accreditation rules or a substitute for local employer, safeguarding, emergency, privacy or clinical procedures. NIC should verify each source and insert Nigeria-specific contacts and requirements before publication.

25. Quality assurance review

Quality question	Finding	Action before launch
CPD value	Develops communication, judgement, boundaries, observation and escalation beyond basic care.	Pilot with working caregivers and supervisors.
Non-duplication	Complements Dementia Care CPD and does not centre dementia diagnosis or management.	Cross-link rather than repeat dementia lessons.
Person-centred care	Emphasises identity, dignity, culture, values, choice and belonging.	Review examples for local fit.
Communication	Provides active listening, validation, reflection, silence and practical scripts.	Assess role-play where possible.
Mental-health awareness	Teaches patterns and change without diagnosis.	Add local referral pathway.
Safeguarding	Includes abuse, coercion, self-harm, suicide and crisis escalation.	Insert current local contacts and procedures.
Scope	Clearly excludes diagnosis, therapy, medication and independent crisis management.	Require scope acknowledgement.
Boundaries	Covers money, gifts, messages, dependency, dual relationships and supervision.	Align with employer code of conduct.

Confidentiality	Includes need-to-know sharing and digital privacy.	Align with current privacy policy and Nigerian requirements.
Cultural relevance	Recognises Nigerian family, faith, language, diaspora and community contexts without stereotyping.	Conduct Nigerian educator review.
Assessment quality	28/35 final items test communication, safety, boundaries, safeguarding and escalation.	Retain reasoning feedback.
Practical value	Provides scripts, checklists, escalation guide and documentation template.	Make job aids downloadable.
Professional credibility	Includes competency framework, scenario assessment, verification, certificate limitations and review period.	Obtain formal NIC approval.
Safety integrity	No methods or detailed self-harm information are included.	Keep crisis content staff-facing and procedural.

Final implementation checklist

The NIC portal should follow **COURSE → MODULES → LESSONS → MODULE ASSESSMENTS → FINAL ASSESSMENT → CERTIFICATE**. Every lesson should display objectives, core content, scenario, professional risks, key practice points and assessment. The portal should provide the five job aids.

Before launch, NIC should add the current emergency number, safeguarding lead, supervisor, mental-health referral route, crisis pathway, complaints route, privacy contact and alternative route where the primary service is unavailable. These must be verified for each delivery setting.

The intended professional standard is:

I understand that professional care includes emotional and social wellbeing. I can listen respectfully, provide appropriate support, recognise meaningful changes, encourage connection and independence, maintain professional boundaries, protect confidentiality, recognise serious concerns and know when to escalate.